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POLICY AT A GLANCE | 01

Bridging the Mental Health Gap: A Multi-level Perspective

Health Sector in India

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Policy at a glance

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The '**Policy-at-a-Glance**' provides users with curated information for first-level analysis and inter-state comparisons of policy. The document brings together all topic-relevant national and state-level acts, policies, and schemes in one place. Further, it identifies for a reader what policy features are covered in which document in a simple table.

The first section of this document sets out some '*Context and Background*' on the topic; the subsequent sections explain the '*Policy Features*' that have been compared, and the last section is the '*Table of Insights*' that marks out what '*Policy Feature*' has been covered (and on which page) in each of the topic-relevant documents.

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1. Context

According to the World Health Organisation (WHO), Mental health is not just the absence of mental disorder but is a state of well-being that enables people to cope with normal stresses of life, realize their abilities, learn well, work well, and contribute positively to their community (WHO, 2022). It is a fundamental aspect of overall life satisfaction, allowing individuals to find meaning in life and actively participate as creative and engaged members of society.

However, in India, mental health and psychosocial well-being have lagged and remained overlooked despite the presence of policies and programmes at the centre level. According to the National Mental Health Survey of 2016, nearly 15 Crores citizens in our country require care and support for their mental well-being. Alarming, between 70 to 92% of these individuals do not receive timely quality treatment for various mental disorders (National Institute of Mental Health and Neuro Sciences, 2016). Self-harm among adolescents is alarming, underscoring the significance of addressing emotional stress and related issues, which greatly contribute to various physical ailments. Mental health professionals, including psychiatrists, clinical psychologists, counsellors, and allied experts, unanimously agree that early intervention holds the potential to avert many future health conditions.

There is a huge shortage of mental health professionals in India, with only about 0.75 psychiatrists per 1,00,000 population compared to the recommended 3 per 1,00,000. This scarcity extends to clinical psychologists, psychiatric social workers, and psychiatric nurses, creating a significant demand-supply gap, resulting in patients bearing substantial costs due to the limited availability of mental health services.

The Rehabilitation Council of India Act 1992 and The National Commission for Allied and Healthcare Professions Act, 2021, outline the need and educational framework for Allied Health Professionals, including mental health professionals. These laws establish crucial guidelines, and standards essential for nurturing mental health professionals within the broader healthcare spectrum.

The National Mental Health Policy (NMHP), introduced in 2014, coupled with the Mental Healthcare Act (MHCA) of 2017, and National Health Policy (NHP) of 2017 outlines strategies aimed at enhancing mental health and upholding individuals' rights.

The National Mental Health Programme (NMHP) was formulated in 1982 to enhance access to basic mental healthcare. The following were the objectives of NMHP:

- i. To ensure the availability and accessibility of minimum mental healthcare for all in the foreseeable future, particularly to the most vulnerable and underprivileged sections of the population.
- ii. To encourage the application of mental health knowledge in general healthcare and in social development.
- iii. To promote community participation in mental health service development and to stimulate efforts towards self-help in the community (NHM, Updated 2023).

Over time, the NMHP programme has undergone significant strategic revisions, including the incorporation of the District Mental Health Programme (DMHP) and the integration of the same with the National Rural Health Mission for effective programme expansion (NHM, Updated 2023). Continuous efforts are underway to bridge the treatment gap by enhancing mental health services and minimizing disparities for people. For example, Ayushman Bharat, which aims to convert primary healthcare facilities into health and wellness centres (HWCs) has integrated provisions for screening mental health conditions.

This document collates various policy approaches aimed at addressing mental health concerns in India. As the landscape of mental health challenges continues to evolve, state governments play a significant role in shaping policies that mitigate the impact and foster recovery. It is important to recognise the diverse strategies adopted by different states which are essential to implement effective approaches and expedite the progress toward improved mental well-being.

Policy Features

The implementation of various specific features about mental health occurs on a state-wide basis, thereby yielding divergent levels of activity coverage across different regions of India. However, it is worth noting that activities related to Mental Health are largely governed by central policies and programmes. This section identifies a common set of policy features and aspects that document referred to in section 3 (table of insights) cover. These policy features can be classified under three broad aspects of policies - 1) Care Centres, 2) Tech enabled care delivery and 3) Community participation and IEC activities.

1) Care Centres

a. Mental Health Day Care Centres

To offer a structured and supportive environment for individuals dealing with mental health challenges, day care centres provide an array of therapeutic interventions, counselling, group activities, and skill development programmes during the day. Their goal is to enhance the mental health and functioning of individuals while allowing them to return to their homes and communities in the evenings, promoting community-based care and reducing the need for institutionalization. For example, the Government of Karnataka has established “Manasadhara Centres” in 15 districts providing day care services for individuals recovering from mental illnesses (Mental Health Programme in Karnataka, 2023). Similar facilities are also offered by the states of Kerala, Madhya Pradesh, and Manipur.

b. Residential Care Activities for Intellectual Disability Patients:

Residential Care Activities for intellectually disabled patients encompasses specialized facilities that provide 24/7 care, housing, and tailored support services to these individuals and aims to enhance their quality of life, social integration, and overall well-being. These facilities are a crucial lifeline for individuals and families dealing with the unique challenges posed by intellectual disabilities. For instance, in the state of Tamil Nadu, the government under Tamil Nadu Mental Health State Policy provides psychiatric rehabilitation facilities and centres for intellectually challenged individuals and mentally ill patients (Tamil Nadu Government Gazette, 2002). Similarly, Karnataka through its Karnataka Mental Health Programme offers residential care activities to intellectually disabled people.

2) Tech Enabled Care Delivery

c. Tele-Mental Health Services:

Tele-mental health services leverage technology to make it possible for individuals to access consultations, counselling, and expert guidance for mental health without being limited by geographic constraints. Through video calls, phone consultations, or online platforms, mental health professionals offer assessments, therapy sessions, and guidance, enhancing accessibility and reducing delays in receiving mental health care. For example, the Government of Tamil Nadu has launched “MANAM” initiative to strengthen the mental health system in educational institutions and provide tele-counselling services to assist those in need (SUBRAMANIAN, 2023). Similarly, the states of Karnataka and Chhattisgarh have also established their own tele mental health helplines, ensuring the access to mental health guidance is accessible in their respective states.

3) Community Participation and IEC Activities

d. Community-Based Mental Health Initiatives:

Community-based mental health initiatives involve integrating mental health services into local communities, making care accessible, and tailored to specific community needs. Community-based initiatives bridge the gap by fostering partnerships with local healthcare providers, engaging community members, and offering mental health services within familiar environments. These initiatives are vital for early detection, reducing stigma, and providing support to individuals in need, ultimately contributing to improved mental health outcomes at a national level. For example, the government of Chhattisgarh in collaboration with NIMHANS, has established Chhattisgarh community mental healthcare tele-mentoring programme (CHaMP) to train Primary Care Doctors (PCDs) and Rural Medical Assistants

(RMA) in screening, identifying, and managing or referring cases of mental health disorders, thereby contributing to the overall well-being of the community and reducing the prevalence of mental illnesses (NIMHANS, 2023). States like Tamil Nadu, Karnataka and Kerala have also launched community based mental health programmes and initiatives, aimed at delivering mental health services.

e. Mental Health Awareness/Literacy Campaigns:

Mental health awareness and literacy campaigns are aimed at educating the public about mental health, eradicating stigma, and promoting understanding and empathy towards mental health issues. These campaigns utilize various platforms, including mass media, community events, workshops, and social media, to disseminate accurate information, encourage early intervention, and create a supportive environment for those affected by mental health challenges. Various states like Karnataka, Tamil Nadu, and Meghalaya undergo extensive mental health awareness campaigns, involving collaborations with mental health professionals, grassroots organisations, and community leaders. These campaigns use multimedia strategies to reach diverse populations and encourage open discussions about mental health to reduce misconceptions and discrimination.

To sum up, the achievements of Karnataka and Tamil Nadu in implementing the nuances of state-level mental health policy, programmes, and initiatives are noteworthy as compared to the progress made by other states of India. These achievements reflect their commitment to covering the essential features necessary for the advancement of mental health services.

3. Table of Insights: Policy Initiatives for Mental Health

Note 1: Documents linked in the tables include original Acts, Policies, Schemes, Programmes and Guidelines. It is aimed at highlighting and enabling a comparison of relevant policy features covered in the documents, across the centre and the states. While all important documents have been identified, the coverage may not be exhaustive. Further, the 'Policy-at-a-Glance' is not aimed at analysing nuances of policies.

Note 2: Page numbers specified in the Table of Insights are as per the PDF pagination opened in a Google Chrome browser. It is possible to encounter differences in the actual page number for some of the documents when opened in a different web browser/application.

Geography	Data				Policy	Care Centres		Tech Enabled Care Delivery	Community Participation and IEC Activities	
	DALY rates (2016) for Depressive Mental Disorders (The India State-Level Disease Burden Initiative Report-2017)	DALY rates (2016) for Anxiety Disorders (The India State-Level Disease Burden Initiative Report-2017)	Burden and Treatment gap of mental Health disorders* (National Mental Health Survey (2015-2016))	Percentage of Districts covered by DMHP* (National Mental Health Survey (2015-2016))	Reference Document	Residential care activities for intellectual disability patients	Mental Health Day care centres	Tele-Mental health services	Community-based mental health initiatives	Mental Health Awareness/Literacy Campaigns
Centre	581	310	NA	NA	National Mental Health Policy	Yes	Yes	Yes		
					Tele Manas			Yes		
					24x7 Toll-Free Mental Health Rehabilitation Helpline, Kiran			Yes		
					Manodarpan				Yes	Yes

Geography	DALY rates (2016) for Depressive Mental Disorders (The India State-Level Disease Burden Initiative Report-2017)	DALY rates (2016) for Anxiety Disorders (The India State-Level Disease Burden Initiative Report-2017)	Burden and Treatment gap of mental Health disorders* (National Mental Health Survey (2015-2016))	Percentage of Districts covered by DMHP* (National Mental Health Survey (2015-2016))	Reference Document	Residential care activities for intellectual disability patients	Mental Health Day care centres	Tele-Mental health services	Community-based mental health initiatives	Mental Health Awareness/Literacy Campaigns
Karnataka	619	325	NA	NA	Karnataka Mental Health Programme	Yes	Yes	Yes		
					Yuva Spandana Programme				Yes	Yes
Kerala	679	420	86.0%	100.0%	Aswasakiranam		Yes		Yes	
Meghalaya	581	301	NA	NA	Meghalaya Mental Health and social care policy					Yes

Geography	DALY rates (2016) for Depressive Mental Disorders (The India State-Level Disease Burden Initiative Report-2017)	DALY rates (2016) for Anxiety Disorders (The India State-Level Disease Burden Initiative Report-2017)	Burden and Treatment gap of mental Health disorders* (National Mental Health Survey (2015-2016))	Percentage of Districts covered by DMHP* (National Mental Health Survey (2015-2016))	Reference Document	Residential care activities for intellectual disability patients	Mental Health Day Care centres	Tele-Mental health services	Community-based mental health initiatives	Mental Health Awareness/Literacy Campaigns
Tamil Nadu	857	314	94.8%	78.1%	Tamil Nadu registration of psychiatric rehabilitation centres	Yes				
					Health and Family Welfare Department-Policy Note			Yes (Pg. 392 and 393)	Yes (Pg. 392)	Yes (Pg. 396)
					Tamil Nadu state wide crisis line			Yes		

*Note:

1. The data points on burden and treatment gap of mental health disorders and percentage of districts covered by DMHP are available for only for those states that have been covered under National Mental Health Policy Survey (2015-2016).
2. The burden of mental health disorders and treatment gap data is evaluated is captured for severe mental disorders, common mental disorders, alcohol use disorders, tobacco use disorders, epilepsy, and high suicidal risk behaviour by the National Mental Health Survey 2015-2016.
3. Most states follow National Mental Health Programme and District Mental Health Programme and do not hold any standalone policy or programme of their own, hence a smaller number of states are visible in the table above.

- WHO: World Health Organisation
- NIMHANS: National Institute of Mental Health and Neuro Sciences
- NMHP: National Mental Health Programme
- NHM: National Health Mission
- DMHP: District Mental Health Programme
- PCDs: Primary Care Doctors
- RMA: Rural Medical Assistants
- HWC: Health and Wellness Centre
- PMJAY: Pradhan Mantri Jan Arogya Yojana
- MHCA: Mental Healthcare Act
- NHP: National Health Policy

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